

Nursing Education and Regulation in Uganda

Executive summary

Uganda's nursing system is governed by a mixed regulatory, educational, and service-delivery architecture. The legal core is the Uganda Nurses and Midwives Council ¹, a statutory professional regulator under the Ministry of Health ², with responsibilities that include regulating standards of training and practice, regulating professional conduct, registering and enrolling practitioners, licensing practice, and supervising aspects of training quality for public safety. The current public regulatory pathway is: approved training program → assessment/examination → UNMC registration/enrolment interview and documentation review → practising licence → periodic renewal and CPD. A practising licence is required for registered or enrolled practitioners and is valid for three calendar years. ³

The strongest public curriculum evidence currently available online is for the Ministry of Education and Sports competence-based Certificate in Nursing curriculum and teacher's guide (2018), plus official syllabus/content outlines hosted through the Uganda Health Professions Assessment Board ⁴ and legacy Uganda Nurses and Midwives Examinations Board ⁵ domains for Diploma in Nursing, Diploma/Certificate in Comprehensive Nursing, Diploma in Mental Health Nursing, Diploma in Public Health Nursing, and related programmes. A publicly retrievable single national BSN curriculum was not found; instead, universities publish institution-specific BSN structures. Among the clearest official university pages, Mbarara University of Science and Technology ⁶ publishes a 4-year Bachelor of Nursing Science with detailed module credits and entry requirements, while Kampala International University ⁷ publishes both direct-entry and extension BSN structures. ⁸

From a platform-design perspective, the main opportunity is not to replace national curricula but to make them usable. Uganda's public source landscape is fragmented across UNMC, UHPAB, Ministry portals, legacy UNMEB files, and institution sites. Some regulator pages are stable and useful, but some UNMC subpages are cluttered by irrelevant spam content or legacy formatting, which makes curriculum discovery and compliance mapping unnecessarily hard. A high-value learning platform should therefore become a structured layer above the official ecosystem: curriculum-mapped content, version-controlled module pages, skills checklists, evidence-linked assessments, clinical simulation, CPD logging, and institution/regulator dashboards. ⁹

Workforce evidence points to both strong supply and severe deployment inefficiency. A 2023 health labour market analysis found an estimated 158,932 health workers in Uganda in 2022, including 106,915 nurses and midwives, who represented 67.3% of the workforce. Uganda had 25.9 doctors, nurses, and midwives per 10,000 population in 2022, still below the SDG benchmark of 44.5 per 10,000. The same analysis found sharp geographic inequity, with Kampala far better supplied than several rural districts, and it also suggested that large shares of registered nursing and midwifery stock were outside the health sector or unemployed when measured against active employment datasets. For an education platform, this means employability, transition-to-practice, licensure support, and rural-ready competency training should be treated as core products, not add-ons. ¹⁰

A pragmatic platform strategy is to launch with a “compliance-first” foundation: entry-to-practice pathways for Certificate in Nursing, Diploma in Nursing, Diploma in Comprehensive Nursing, Diploma in Midwifery, Diploma in Mental Health Nursing, Diploma in Public Health Nursing, and Bachelor of Nursing Science; then layer in skills labs, clinical case banks, CPD modules, tutor tools, student logbooks, and speciality tracks. Priority partnerships should be with UNMC, UHPAB, leading public and faith-based schools, BSN-granting universities, and CPD providers. ¹¹

Regulatory and professional framework

UNMC’s public materials and the Nurses and Midwives Act present a coherent regulatory mandate even though the online materials are spread across several pages. The Council’s functions include regulating standards of nursing and midwifery, regulating conduct, and maintaining the professional system of registration, enrolment, and licensing. UNMC’s own public pages describe it as the body that regulates nursing and midwifery professionals in the country, while the legal text confirms the Council’s statutory functions. A 2025 UNMC job advert also shows that compliance inspection of health training institutions, curriculum review, and pre-licensure test-item setting remain part of its operational role. ¹²

The licensing pathway is more explicit than the curriculum pathway. UNMC states that all nurses and midwives working in Uganda must have a general licence to practise, renewed every three years, and that first registration/enrolment fees include the first three years of licensure. The legal text indexed by ULII also states that the practising licence is valid for three calendar years and renewable subject to Council conditions. UNMC’s 2023 public notice confirms that registration and renewal are now online, via the Council portal, and lists the document package needed for renewal, including the expired licence, academic certificates, passport photo, secondary-school result slips/certificates, UNMEB certificates where applicable, and university transcript/certificate for degree holders. ¹³

For locally trained applicants, UNMC’s public guidance still centres on examination results, booking for interview with the Council, and presentation of original documents. This matters for platform design because “student success” in Uganda does not end at final exams. The platform should therefore include a licensure-readiness layer: document vault, interview preparation, checklist validation, result-to-registration workflow, and alerts for renewal dates. ¹⁴

UNMC’s public professional-ethics page anchors scope of practice around four duties under section 24 of the Act: promote health and wellness, prevent illness, restore health, and alleviate suffering. It also frames professional accountability to people served, practice, profession, and co-workers. A stable public copy of the more detailed national scope-of-practice document was not retrieved in this research run, although UNMC’s homepage clearly indicates that a scope-of-practice launch occurred. For platform alignment, the safest current approach is to map content to three official layers that are public and stable enough to use: Act-based obligations, curriculum competency matrices, and CPD-approved practice domains. ¹⁵

The CPD framework is operational but public messaging is not perfectly consistent. One UNMC 2024 article states that every nurse or midwife is expected to attain a minimum of 50 credit units in a period of three years before licence renewal, and that the Council had accredited more than 13 CPD providers and more than 20 CPD programmes. A second 2024 UNMC article says credits were “not yet mandatory” for renewal, while also explaining that the guidelines require participation and that providers can receive five-year accreditation. Because both statements are recent and official, the most responsible conclusion for a platform is conservative: build a credit-tracking system around a 50-credit/3-year architecture, but advise

users and institutional partners to verify the live renewal rule with UNMC before using the platform for high-stakes compliance decisions. ¹⁶

The current cadre structure recognized publicly by UNMC includes enrolled, registered, degree, specialist, education, and graduate pathways. Public UNMC and Ministry documents list Enrolled Nurse, Enrolled Midwife, Enrolled Mental Health Nurse, Registered Nurse, Registered Midwife, Registered Comprehensive Nurse, Registered Mental Health Nurse, Registered Paediatric Nurse, Registered Public Health Nurse, Registered BSN, Registered BSc Midwifery, Registered Master of Science Nursing, and health-tutor categories. Ministry schemes of service also show diploma, advanced/higher diploma, degree, and postgraduate diploma strata in nursing and midwifery. ¹⁷

flowchart LR

A[Approved training programme] --> B[Assessment or examination]

B --> C[UNMC registration or enrolment]

C --> D[Practising licence]

D --> E[Practice and CPD]

E --> F[Licence renewal]

F --> G[Progression to specialty, degree completion, teaching, leadership, or private practice]

The pathway above reflects the live public structure across UNMC pages, legal references, and current online renewal notices. ¹⁸

Education pathways and curricula

The strongest publicly accessible national curriculum in nursing is the 2018 Certificate in Nursing curriculum issued by the Ministry of Education and Sports ¹⁹. It requires Uganda Certificate of Education or equivalent, minimum passes in English, Biology, Chemistry, Physics, and Mathematics, an oral interview, and admission into the programme as a choice. The programme duration is two and a half years full-time. The same curriculum states that field placement totals four weeks, with two weeks in community health and two weeks in mental health, and that the practicum contributes 10% of coursework assessment. Assessment is competence-based: 40% continuous assessment and 60% end-of-semester examination, with continuous assessment covering lab/practical work, assignments, tests, clinical assessment, and clinical record books. ²⁰

The Certificate in Nursing curriculum is unusually useful for platform design because it exposes both competencies and assessment logic. Its teacher's guide says graduates should be able to admit patients, take vital observations, carry out general nursing care, provide specific nursing care, administer medicine, prepare for ward rounds, prevent and control infection, and prepare patients for procedures. The curriculum alignment matrix further maps tasks to domains such as nursing care, medical nursing, surgical nursing, mental health nursing, paediatric nursing, gynaecological nursing, community health nursing, occupational health, and health-unit management. This is a strong foundation for a modular digital course architecture. ²¹

Official diploma and specialty syllabi are currently easier to access through the UHPAB/legacy UNMEB ecosystem than through a single Ministry curriculum repository. The published Diploma in Nursing direct

syllabus shows a three-year, six-semester structure by inference from sequential year/semester content blocks, beginning with anatomy and physiology, first aid, foundations of nursing, sociology, psychology, introductory computing, microbiology, personal and communal health, then moving into medical nursing, pharmacology, surgical nursing, paediatrics, and later advanced adult-care content. The Diploma in Nursing extension syllabus focuses on post-basic upgrading content such as Foundations of Nursing III, specialized procedures, advanced medical nursing, ophthalmology, peri-operative care, and orthopaedic nursing; institution pages from KIU and IUIU publicly describe extension diplomas as 1.5 years. ²²

The public syllabi also confirm recognized specialty and parallel pathways. Diploma in Mental Health Nursing covers anatomy and physiology, foundations of nursing, computer applications, microbiology, sociology, psychology, principles of psychiatric nursing, medical-surgical nursing, psychopharmacology, paediatric nursing, child psychiatry, and later-year mental-health content. Diploma in Public Health Nursing includes public health nursing, health-services management, primary health care, immunization systems, counselling, communication, and entrepreneurship-oriented management content. Certificate and Diploma in Comprehensive Nursing syllabi are also publicly listed, although in the retrieved PDFs they point back to older Ministry curricula rather than reproducing complete curriculum text. ²³

For the BSN level, public access shifts from national curriculum to university-level programme design. MUST publishes a 4-year Bachelor of Nursing Science, with direct entry requiring two principal passes in Biology and Chemistry and at least a subsidiary pass in either Physics or Mathematics. Its diploma-entry route accepts registered nurses, midwives, comprehensive nurses, psychiatric nurses, or paediatric nurses eligible for UNMC registration, plus O-level science passes and an entrance examination in anatomy, physiology, microbiology, and general paper. MUST also publishes the module list, which spans anatomy, biochemistry, biostatistics, communication, nursing ethics, health assessment, pathophysiology, pharmacology, midwifery, medical-surgical nursing, paediatric nursing, psychiatric nursing, community nursing, leadership and community placement, nursing education, administration and management, advanced nursing practice, critical care, geriatric nursing, disaster preparedness, and dissertation. Summing the published module credits yields about 193 credit units; that arithmetic is an inference from the official module list, not a number explicitly stated by the university. ²⁴

KIU's direct-entry BSN is officially described as an 8-semester, 4-year programme with two recess semesters for clinical skills in health facilities and no classroom teaching during those recess terms. Its publicly listed structure includes nutrition, nursing research, health assessment and nursing process, clinical nursing skills, reproductive health, medical-surgical nursing, child health nursing, mental health nursing, professional nursing practice, and specialized medical-surgical content. KIU's extension BSN is publicly listed as a 3-year weekend programme. UCU's 2026 course list confirms both direct-entry and diploma-completion Bachelor of Nursing Science pathways. Makerere's published diploma-holder requirements further confirm degree-upgrading routes into BSN for holders of recognized health diplomas. ²⁵

Programme comparison

Programme	Main official public source	Entry requirements	Duration	Core learning emphasis	Clinical / practicum detail	Assessment / awarding body
Certificate in Nursing	MoES Curriculum for Certificate in Nursing 2018	UCE/ equivalent; passes in English, Biology, Chemistry, Physics, Mathematics; oral interview	2.5 years full-time	Nursing care, med-surg, paediatrics, mental health, gynae, community health, health-unit management	4 weeks total field placement: 2 community, 2 mental health	40% continuous, 60% end-semester; exams administered by UNMEB ²⁰
Diploma in Nursing Direct	UHPAB/ legacy UNMEB syllabus	Public syllabus does not restate admission criteria; institutions typically use certificate/ background-based routes	3 years by inference from year/ semester structure	Anatomy, first aid, foundations, sociology, psychology, computer, microbiology, personal/community health, adult nursing, pharmacology, surgical and paediatric nursing	Public syllabus retrieved does not specify hours	Assessment details not stated in retrieved syllabus; UHPAB hosts syllabus index, legacy UNMEB PDF holds content outline ²⁶
Diploma in Nursing Extension	UHPAB/ legacy UNMEB syllabus plus university extension pages	Post-basic upgrade route; institution-specific	1.5 years on official university extension pages	Advanced procedures, specialized nursing care, advanced medical nursing	Public syllabus retrieved does not specify hours	Institution/ programme-specific; UHPAB hosts syllabus index ²⁷
Certificate in Comprehensive Nursing	UHPAB/ legacy UNMEB syllabus	Not restated in retrieved syllabus	Unspecified in retrieved syllabus	Anatomy, first aid, foundations, socio-psychology, computer, nursing basics	Unspecified	Official syllabus refers back to older MoES curriculum for topic detail ²⁸

Programme	Main official public source	Entry requirements	Duration	Core learning emphasis	Clinical / practicum detail	Assessment / awarding body
Diploma in Comprehensive Nursing Direct	UHPAB/ legacy UNMEB syllabus	Not restated in retrieved syllabus	Unspecified in retrieved syllabus	Anatomy, first aid, foundations, microbiology, socio-psychology, direct practicals	Public syllabus includes “practicals” but not hour totals	Official syllabus refers back to MoES 2004 curriculum for detail ²⁹
Diploma in Mental Health Nursing	UHPAB/ legacy UNMEB syllabus	Not restated in retrieved syllabus	3 years by inference from year/ semester structure	Psychiatric nursing, psychology, psychopharmacology, child psychiatry, med-surg, paediatrics	Unspecified in retrieved syllabus	Official syllabus content guideline on legacy UNMEB domain ³⁰
Diploma in Public Health Nursing	UHPAB/ legacy UNMEB syllabus	Not restated in retrieved syllabus	Unspecified in retrieved syllabus	Public health nursing, management, PHC, EPI, community assessment, leadership	Unspecified in retrieved syllabus	Official syllabus content guideline on legacy UNMEB domain ³¹
Bachelor of Nursing Science	MUST and university programme pages	Direct: A-level science combination; diploma entry available for registered cadres at some universities	4 years direct; 3 years extension at KIU	Foundational sciences, ethics, assessment, med-surg, midwifery, child health, mental health, community, leadership, education, administration, research	KIU states recess semesters are for clinical skills; hour totals generally unspecified in public pages	University-awarded degree; regulator recognition via UNMC recognized-schools list ³²

Recognized qualifications and career ladder

Uganda publicly recognizes a broader nursing ladder than only certificate, diploma, and BSN. Ministry schemes of service and UNMC public pages show a progression from enrolled to registered cadres, then higher diplomas, degrees, and postgraduate/teaching routes. Recognized or publicly referenced tracks include nursing, comprehensive nursing, midwifery, mental health nursing, paediatric nursing, public health nursing, critical care nursing, oncology nursing, palliative care nursing, health-tutorship/medical

education, BSc Midwifery, Master of Nursing Science, and postgraduate diploma-level development. Where a public curriculum or syllabus PDF could not be retrieved in this research run, it is marked as unspecified rather than assumed. ¹⁷

Institutions, resources, and open materials

UNMC's Recognized Schools directory is the authoritative live list for nursing and midwifery training institutions because it includes programmes offered, attached training health facility, district, contact phone/email, and registration status. A second useful official list is the Ministry of Education and Sports PDF on health training institutions offering diploma and certificate programmes. Together, these two sources are the best current backbone for building an institution directory inside a learning platform. ³³

The government nursing and midwifery training institutions listed by UNMC include Arua School of Nursing, Butabika School of Psychiatric Nursing, Hoima School of Nursing and Midwifery, Jinja School of Nursing and Midwifery, Kabale School of Enrolled Comprehensive Nursing, Lira School of Comprehensive Nursing, Masaka School of Comprehensive Nursing, Mulago Health Tutors College, Mulago School of Nursing and Midwifery, Public Health Nurses College, and Soroti School of Comprehensive Nursing. ³⁴

The faith-based recognized institutions include Bugongi College of Nursing and Midwifery, Fort Portal International Nurses Training School, Ibanda School of Nursing and Midwifery, Ishaka Seventh Day Adventist School of Nursing, Kagando School of Nursing and Midwifery, Kalongo School of Nursing and Midwifery, Kalungi School of Nursing and Midwifery, Kamuli School of Nursing and Midwifery, Kibuli School of Nursing and Midwifery, Kisiizi School of Nursing, Kiwoko School of Nursing, Kuluva School of Nursing and Midwifery, Lacor School of Nursing and Midwifery, Matany School of Nursing and Midwifery, Mbale School of Nursing and Midwifery, Mengo School of Nursing and Midwifery, Mukono Diocese School of Nursing and Midwifery, Ngora School of Nursing, Nsambya School of Nursing and Midwifery, Lubaga Health Training Institution, Rugarama School of Nursing, Saleem School of Nursing and Midwifery, St. Francis Nyenga School of Nursing, St. Karoli Lwanga Nyakibale School of Nursing and Midwifery, Uganda Nursing School Bwindi, Villa Maria School of Nursing, and Virika School of Nursing and Midwifery. The same official page records whether each has full registration or provisional licence. ³⁵

UNMC's private section includes Access School of Nursing and Midwifery, Agule School of Nursing and Midwifery, Alice Anume Memorial School of Nursing and Midwifery, Bweyale School of Nursing and Midwifery, DAF School of Nursing and Midwifery, Good Samaritan School of Nursing and Midwifery, Gulu School of Nursing and Midwifery, King James School of Nursing and Midwifery, Kumi School of Nursing and Midwifery, Lubega Institute of Nursing and Medical Science, Maracha School of Nursing and Midwifery, Masaka School of Health Sciences, Mityana Institute of Nursing and Midwifery, Mutolere School of Nursing and Midwifery, Ntungamo Health Training Institute, Iganga School of Nursing and Midwifery, Indian Institute of Health and Allied Sciences, International Institute of Health Sciences, Jerusalem Institute of Health Sciences and Technology, Johnass International College of Health Sciences, Kabale Institute of Health Sciences, Kyetume School of Nursing and Midwifery, Lugazi School of Nursing, Mayanja Memorial Medical Training Institute, Florence Nightingale School of Nursing and Midwifery, Rakai Community Nursing School, Rwenzori School of Nursing and Midwifery, St. Eliza School of Nursing and Midwifery, St. Gertrude School of Nursing and Midwifery, St. John School of Nursing, St. Joseph's Midwifery and Nurses Training School, Tumu Medical Institute, Uganda Christian Institute for Professional Development, and Uganda Martyrs School of Nursing and Midwifery. ³⁵

UNMC also recognizes university-level providers, including Makerere University ³⁶, Busitema University ³⁷, MUST, Uganda Christian University ³⁸, KIU, Bugema University, Bishop Stuart University, International Health Sciences University, Kampala University, Mountains of the Moon University, The Aga Khan University, Victoria University, and IUIU-linked nursing school provision. Their recognized offers include BNSc, BSc Midwifery, Master of Nursing Science tracks, and some diploma routes. ³⁹

Selected official programme pages and downloadable documents

Priority	Resource	Why it matters	Format
High	Curriculum for Certificate in Nursing 2018 ⁴⁰	Best publicly available national nursing curriculum; includes admission, duration, assessment, competency matrix, sample assessments	PDF
High	Teacher's Guide for Certificate in Nursing 2018 ⁴¹	Most practical public teaching guide; useful for platform pedagogy, lesson flow, and tutor tools	PDF
High	UHPAB Exam Syllabus portal ⁴²	Current official index to diploma and specialty syllabus documents	HTML index
High	Diploma in Nursing Direct syllabus ⁴³	Official content outline for direct diploma	PDF
High	Diploma in Mental Health Nursing syllabus ³⁰	Official specialty syllabus content	PDF
High	Diploma in Public Health Nursing syllabus ³¹	Official public-health nursing content outline	PDF
High	UNMC Recognized Schools directory ⁴⁴	Official institution list with programmes, contacts, districts, and registration status	HTML
High	Health Training Institutions in Uganda offering diploma and certificate programmes ⁴⁵	Ministry-level directory for diploma/certificate providers	PDF
High	Bachelor of Nursing Science at MUST ²⁴	Clear official BSN structure, entry requirements, and detailed modules	HTML
Medium	KIU BSN direct and extension pages ⁴⁶	Good official public BSN structure examples, including recess clinical semesters and extension pathway	HTML
Medium	Schemes of Service for the Nursing and Midwifery Cadre ⁴⁷	Best public document for career progression and higher/specialist tracks	PDF

Priority	Resource	Why it matters	Format
Medium	UNMC CPD pages and CPD provider page ¹⁶	Core source for CPD architecture, provider accreditation, and renewal logic	HTML/ PDF- linked
Medium	Analysis of the Health Labour Market of Uganda 2023 ⁴⁸	Strongest recent workforce analysis for planning, market sizing, and equity targeting	PDF
Medium	Approved Structure for Health Centre IV and III, April 2023 ⁴⁹	Essential for clinical-role mapping and simulation priorities at lower-level facilities	PDF
Medium	UNMC public notice on online renewal requirements ⁵⁰	Useful for compliance and onboarding workflows	PDF

Publicly downloadable lecture slides, checklists, and rubrics for Ugandan nursing schools are much scarcer than curricula and syllabi. The strongest official substitutes currently online are the MoES teacher's guide, certificate curriculum assessment instruments, the UHPAB syllabus set, and training-platform or e-learning shells published by some schools. A public Moodle-style course shell is visible at Kuluva School of Nursing for several diploma nursing modules; it is not a complete official repository, but it shows that schools are beginning to post course-level digital content. ⁵¹

For open educational resources that can legitimately supplement Ugandan content, the most useful resources found in this research run were the Jhpiego Helping Mothers Survive implementation guide, which explicitly states it can be integrated into pre-service education; the UNMC/WCEA online CPD ecosystem; and IntraHealth's digital-library work in Ugandan health training institutions. These are not replacements for Uganda-specific curricula, but they are practical sources for maternal/newborn emergency drills, digital reading access, and tutorial support. ⁵²

Workforce, employers, and ecosystem

A recent labour-market analysis found that Uganda had an estimated 158,932 health workers in 2022, of whom 106,915 were nurses and midwives. That made nursing and midwifery the dominant health workforce grouping at 67.3% of the total. The same analysis estimated nursing and midwifery density at 23.85 per 10,000 population, and combined doctors, nurses, and midwives at 25.9 per 10,000, which is about 58% of the SDG benchmark of 44.5 per 10,000. ⁵³

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pie showData
  title Uganda health workforce stock in 2022
  "Nurses and midwives" : 106915
  "Laboratory technicians" : 16098
  "Clinical officers" : 13627
  "Medical doctors" : 7793
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"Pharmacists" : 1712

"Dentists" : 504

The chart reflects the 2023 labour-market analysis of Uganda's 2022 workforce stock. ⁵⁴

Distribution is the bigger problem than production. The equity analysis found Kampala with the highest density/geographic equity index for doctors, nurses, and midwives, followed by Moroto, Butambala, Kabale, and Lyantonde, while Kibaale, Buyende, Kyenjojo, Kakumiro, and Kyegegwa had the lowest index values. This is an explicit signal that any learning platform should include offline-first delivery, rural preceptor support, and district-sensitive deployment models rather than assuming urban training conditions. ⁵⁵

Employment evidence is mixed but operationally important. Among employed health workers, the labour-market report says 74% of nursing professionals and 64% of midwifery professionals were in the public sector, with the balance in private provision. But when the analysis looked at total registered stock against available employment data, it estimated that only 25% of nursing professionals were in the public sector, 9% in private sectors, and 66% were outside the health sector or unemployed; for midwifery professionals the corresponding numbers were 20%, 11%, and 68%. These two findings are not contradictory: one compares public-versus-private shares among those employed, while the other compares employment against total stock. For platform design, that means the market is not just students and licensed clinicians already in jobs. It also includes underemployed, transitioning, upgrading, and licence-renewing nurses. ⁵⁶

The public service staffing picture also depends on what standard is used. The 2022/23 annual health sector performance report said the target for physicians, nurses, and midwives in the country had been achieved, although public-sector staffing was still low at 74%. A later 2023/24 report snippet indicates staffing in public health facilities was 34% based on new structures. The most plausible interpretation is that Uganda's revised staffing structures are more ambitious than older norms, which makes the gap appear much larger when measured against the new establishment. That matters for an online platform: there is growing demand not just to produce workers, but to make each worker more productive through delegation, refreshers, simulation, and low-dose high-frequency skills reinforcement. ⁵⁷

The biggest employers for nurses and midwives are public facilities and publicly affiliated teaching sites: national referral hospitals, specialized institutes, regional referral hospitals, district/general hospitals, and health centres. In Greater Mulago alone, UNMC's 2024 validation exercise referenced major employing institutions such as Mulago Women's and Neonatal Specialized Hospital ⁵⁸, Mulago National Referral Hospital ⁵⁹, Uganda Heart Institute ⁶⁰, (organization, Uganda Cancer Institute, kampala uganda), Makerere's nursing and health projects, and other Kampala-based academic/NGO health employers. The same exercise validated 1,171 nurses and midwives, of whom 1,111 had valid practising licences. ⁶¹

Faith-based and private-not-for-profit networks are also major employers and training partners. Uganda Catholic Medical Bureau ⁶² states that its network includes 33 hospitals, 263 lower-level units, and 15 health training institutions/schools. The labour-market analysis also used private-not-for-profit and Uganda Medical Bureaus data in estimating effective supply. For a platform, this means partnerships with PNFP networks can expand clinical placements, tutor networks, rural case libraries, and CPD delivery far beyond the public-school system alone. ⁶³

Professional and ecosystem partners are sufficiently mature for platform partnerships. UNMC is the regulator; UNMU is the main publicly visible labour/professional union and is also listed by the International Council of Nurses ⁶⁴ as Uganda's national association member. On the development side, Seed Global Health ⁶⁵ publicly states that it partners in Uganda to train nurses, midwives, and physicians and deploys nurse educators, including support to bachelor's and master's level pediatric nursing education. Jhpiego ⁶⁶ and IntraHealth International ⁶⁷ have also published Uganda-specific work on health-worker learning, maternal/newborn skills, digital libraries, and national HRH information systems. ⁶⁸

The health-centre structures approved in 2023 are especially revealing for platform content planning. A Health Centre IV establishment includes specialized nursing positions in nursing, midwifery, psychiatry, public health, critical care, palliative care, and large numbers of assistant nursing officers, enrolled nurses, and enrolled midwives. A Health Centre III establishment still includes nursing officers, assistant nursing officers across several specializations, and eight enrolled midwives plus eight enrolled nurses. This means the platform should not design only for degree-level hospital nurses; it must serve mixed-skill teams who work at HC III/IV level and often rotate between maternal, adult, infectious disease, emergency, psychiatry, and community roles. ⁶⁹

Platform design recommendations

The platform should be built as a standards-anchored, role-based, and progression-aware system. The foundational content model should not be “courses by subject” alone. It should be a three-layer map: national duty/competency domain, qualification/programme level, and practice setting. The national duty/competency layer can be derived from the public Certificate in Nursing competency matrix, UNMC's Act-based obligations, public CPD practice areas, and university BSN module structures. The result is a national competency spine that works across certificate, diploma, BSN, CPD, and speciality pathways. ⁷⁰

Suggested module architecture mapped to national competencies

Platform module family	National alignment basis	Target learner levels	Recommended digital assets
Professional regulation, ethics, and patient rights	UNMC ethics page; Act obligations; licensure workflow	All pre-service and in-service	Interactive law scenarios, ethics OSCEs, licence checklists
Foundations of nursing and health assessment	Certificate curriculum; DN syllabus; BSN modules	Certificate, diploma, BSN	Video demonstrations, guided note packs, vitals simulators
Infection prevention, safety, and quality	Certificate curriculum; CPD domains; national training manuals	All levels	SOPs, IPC drills, ward simulations, audit checklists
Medicines, pharmacology, and medication administration	Certificate and diploma syllabi; BSN pharmacology modules	Diploma, BSN, upgrading tracks	Dose-calculation engine, medication OSCE stations, case banks

Platform module family	National alignment basis	Target learner levels	Recommended digital assets
Adult medical-surgical nursing	DN, BSN, HC III/IV staffing mix	Diploma, BSN, CPD	Case-based learning, emergency pathways, procedure videos
Maternal, reproductive, newborn, and midwifery care	Midwifery pathways, CPD domains, HC III/IV structures	Midwifery, nursing, BSN, CPD	Labour and PPH simulations, skills checklists, neonatal drills
Child health and paediatric nursing	DN/DMHN/BSN paediatric content	Diploma, BSN, CPD	Growth-monitoring tools, IMCI pathways, paediatric OSCEs
Mental health and psychosocial care	DMHN syllabus; UNMC scope-related CPD areas	Certificate, diploma, BSN, CPD	Interview practice, de-escalation scenarios, reflective portfolios
Community and public health nursing	Certificate field placement; DPHN syllabus; BSN community nursing	All levels	Outreach planning templates, household/ community case maps
Leadership, management, tutoring, and entrepreneurship	Manage Health Unit, DPHN management content, nursing administration, CPD provider logic	Diploma extension, BSN, CPD, tutors	Shift leadership simulations, mentorship tools, facility dashboards
Research, evidence-based practice, and digital health	BSN research modules; UNMC journal and seed-grant direction	BSN, graduate, tutors	Research methods micro-courses, appraisal tools, manuscript bootcamps
CPD and speciality tracks	UNMC CPD areas and provider system	Licensed practitioners	Credit-bearing microlearning, provider accreditation support, reflective logs

This structure is a synthesis from public Ugandan sources rather than a single official framework, but it is tightly aligned to the duties, modules, and CPD domains that are already public. ⁷¹

The pedagogy should be blended and assessment-rich. Uganda's strongest public curriculum materials are competence-based and practical. The Certificate in Nursing curriculum already specifies written and performance assessment, continuous assessment weighting, clinical assessment, record books, and practical criteria. The platform should mirror that by supporting MCQs, short-answer questions, sequencing items, practical checklists, OSCE rubrics, logbooks, and supervisor sign-off, rather than relying only on video lessons and end-of-course quizzes. ⁷²

Clinical skills simulation should be a first-class feature, especially for lower-level facilities and extension students. The public establishment structures for HC III and HC IV show that many sites need nurses who can cover general nursing, maternity, psychiatry, critical care, palliative care, infection control, health

education, and referral preparation. Platform simulations should therefore prioritize: triage, vitals, drug administration, labour and newborn care, postpartum haemorrhage, sepsis recognition, oxygen therapy, transfusion support, catheterization, wound care, mental-health assessment, community outreach planning, immunization/cold chain basics, and multidisciplinary escalation. ⁷³

CPD should be built in from the start. At minimum, the platform should support verified micro-courses, activity attendance, supervisor sign-off, downloadable certificates, credit aggregation, and facility-level reporting. Since UNMC's public messaging on whether credits are already mandatory for renewal is not completely consistent, the platform should display regulatory-status notices clearly and allow administrators to update rules centrally when UNMC clarifies or changes them. ¹⁶

Partnerships should be prioritized in this order. First, UNMC and UHPAB, because without regulatory and assessment alignment, the platform becomes merely supplementary. Second, the strongest public and faith-based schools, because they control tutor workflows and clinical placement realities. Third, BSN universities such as Makerere, MUST, UCU, KIU, and Busitema. Fourth, PNFP networks such as UCMB. Fifth, development partners already active in educator deployment, maternal/newborn simulation, and digital libraries, especially Seed Global Health, Jhpiego, and IntraHealth. ⁷⁴

flowchart TD

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A[Official source layer] --> B[Curriculum and competency map]
B --> C[Learning objects]
C --> D[Assessments and OSCE rubrics]
C --> E[Clinical simulations]
C --> F[CPD microlearning]
D --> G[Student and tutor dashboards]
E --> G
F --> H[Licence renewal and CPD tracking]
G --> I[School and regulator reports]
```

This architecture directly responds to the fact that Uganda's current source environment is fragmented but rich enough to support a robust standards-based platform if the content is normalized and version-controlled. ⁷⁵

Open questions and limitations

Some important items were not fully retrievable from stable primary public sources in this research run. A stable public copy of the detailed UNMC scope-of-practice document was not retrieved, even though UNMC's site indicates it exists. A single public national BSN curriculum was also not found; what is publicly available are institution-specific BSN structures. Publicly accessible clinical-hour totals are sparse for many diploma and BSN programmes, so this report marks them as unspecified where necessary rather than guessing. ⁷⁶

UNMC's website currently contains some polluted or outdated subpages with irrelevant spam content in otherwise genuine sections such as cadres/downloads. The core regulatory pages, portal notices, and recognized-schools directory are still usable, but any production platform should not scrape UNMC blindly without human curation and version control. ⁷⁷

CPD rules need one final formal confirmation before being hard-coded into any compliance workflow. UNMC's 2024 public materials support a 50-credit/3-year model, but one article presents those credits as necessary before renewal while another says they are not yet mandatory. A platform can still safely build the ledger, reporting, and accreditation logic now, but the legal/administrative switch for "mandatory at renewal" should remain configurable pending confirmation from UNMC. ⁷⁸

The latest public staffing picture should also be interpreted carefully because the reported staffing percentage changes depending on whether older or newer staffing structures are used. For planning, that is not a reason to wait. It is a reason to make the platform flexible enough to serve both pre-service training and in-service productivity support at understaffed facilities. ⁷⁹

¹ ⁵ ⁸ ²⁰ ⁴⁰ ⁶⁴ ⁷⁰ ⁷¹ ⁷² <https://library.health.go.ug/file-download/download/public/250>
<https://library.health.go.ug/file-download/download/public/250>

² ²² ²⁶ ⁴³ <https://unmeb.go.ug/wp-content/uploads/2018/10/EXAM-SYLLABUS-FOR-DN-INTAKE-JULY-2018.pdf>
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³ **Nurses and Midwives Act**
https://ulii.org/akn/ug/act/1996/2/eng%402000-12-31?utm_source=chatgpt.com

⁴ ⁵⁷ ⁷⁹ https://library.health.go.ug/sites/default/files/resources/PRINT_AHSPR%202022.pdf
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⁶ ⁹ ¹² ³⁸ ⁶² ⁶⁵ ⁷⁵ ⁷⁶ <https://unmc.ug/>
<https://unmc.ug/>

⁷ ²⁴ ³² <https://www.must.ac.ug/programme/bachelor-of-nursing-science/>
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¹⁰ ¹⁹ ³⁶ ⁴⁸ ⁵³ ⁵⁴ ⁵⁵ ⁵⁶ https://files.aho.afro.who.int/afahobckpcontainer/production/files/Uganda_HLMA_report-Final_Signed.pdf
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¹¹ ³³ ³⁴ ³⁵ ³⁹ ⁴⁴ ⁶⁶ ⁷⁴ <https://unmc.ug/recognized-schools/>
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¹³ <https://unmc.ug/how-to-obtain-a-license/>
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